Pigmentation science — melasma, PIH, tanning

పిగ్మెంటేషన్ సైన్స్ — మెలాస్మా, PIH, టానింగ్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Differentiate the common pigmentation patterns on Indian skin
- Understand why melasma relapses and how to counsel for it
- Build a pigmentation treatment plan with the doctor

THEORY · థియరీ

Common patterns

Melasma: symmetrical brown patches on cheeks, forehead, upper lip; hormone, heat and light driven; chronic and relapsing. PIH: brown marks after inflammation (acne, injury, waxing). Tan: diffuse darkening on exposed areas, reversible. Periorbital melanosis (dark circles): pigment + shadow + vessels + hollowness — treat only the right component. Freckles/lentigines: sun-induced spots.

Why melasma is different

Melanocytes are hyperactive, there is vascularity and often a dermal component. It flares with sun, visible light, heat (cooking, travel), hormones (pregnancy, OCP), and aggressive treatment. Management is control, not cure: gentle peels, tranexamic-based protocols by the doctor, strict tinted sunscreen, heat avoidance and patience.

Building a plan

Prime 2–4 weeks (barrier + sunscreen + brightening) → gentle peel series every 3–4 weeks → maintenance. Add PICO or Q-switched toning only under the dermatologist, with low fluence. Review at 8 weeks with photographs. Aggressive lasering of melasma in Indian skin makes it worse — this is the single biggest mistake in our market.

Hands-on today

- Identify pigmentation type on 5 clinical photos + 2 live clients
- Use the dermoscope to see epidermal vs dermal pigment patterns
- Draft a 3-month melasma plan and present it to the trainer

Safety & contraindications

- Never promise permanent clearance of melasma
- Any sudden, single, changing dark patch → doctor review (rule out other causes)

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Melasma = control, not cure
- Heat and visible light matter, not just UV
- Tinted sunscreen for melasma
- Low and slow in Fitzpatrick IV–V

Homework

Write a melasma counselling script covering relapse, sunscreen and realistic timelines.

Grooming & professional tip

Give melasma clients a 'flare plan' for summer and travel. Clients who feel supported through a flare stay for years.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్